



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : دنيا عمر محمد عمر
المحافظة : القاهرة
الحى / المدينة : عين شمس
العنوان : //
رقم البطاقة : 28506112100209 رقم التليفون : 01004611684
تاريخ تسجيل الإستمارة : 09-10-2026
العمر (تاريخ الميلاد) : 11-06-1985 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي
الذين يتلقوا علاج هرموني حمض الزيلودرونك [14157]
التعليق على التشخيص : -----
قسم العلاج :
الاورام

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: دنيا عمر محمد عمر National ID: 28506112100209	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

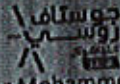
نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Patient's Name: Dunia Omar Mohammed
Date: 2026-05-21
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE NON CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

I- CHEST:

- Known case of breast cancer
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. Two right and left focal lesions the largest seen in the right lobe segment VI measuring 15 x 15 mm showed marginal enhancement. No dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ Known case of breast cancer.
- ❖ Two hepatic focal lesions likely metastatic.

Dr. Asser Ahmed

Dr. Asser Ahmed



٢٠٢٣/٠٨

٢٨٥٠٦١١٢١٠٠٢٠٩

اخصائية تحاليل طبية



متزوجة

مصلحة

أنثى

محمد نور الدين محمد السيد

البطاقة سارية حتى ٢٠٣٠/٠٨/٠٩



دنيا
بطاقة تحقيق الشخصية

دنيا

عمر محمد عمر

١١ ش عبدالرحمن عنبر

الجيزة - الجيزة

١٩٨٥/٠١



٢٨٥٠٦١١٢١٠٠٢٠٩

KC3436581

Name: دنيا عمر محمد MR. : 437	
Detailed Medical Report	
السيد	
نخبة طبية وبعد,,	
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته	
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient is a 41-year-old premenopausal individual with a positive family history of breast cancer in an aunt. No co-morbidities are reported. The clinical course began in 2019 with a diagnosis of premenopausal right-sided breast cancer, treated with breast-conserving surgery (BCS) in June 2019. Pathology at that time showed a T2N0M0 invasive ductal carcinoma, grade III, estrogen receptor (ER) 4/8, progesterone receptor (PR) 5/8, HER2 + 3, and Ki-67 72 %. Adjuvant therapy comprised four cycles of AC followed by 12 cycles of taxol plus 17 cycles of trastuzumab, completed in February 2021, followed by tamoxifen plus zoladex until December 2023. Subsequent disease progression was identified on a biopsy of a hepatic focal lesion (HFL), which demonstrated metastatic adenocarcinoma with ER 4/8, PR negative, HER2 + 3. The patient received nine cycles of taxol plus three cycles of trastuzumab. A new CT chest-abdomen-pelvis performed on 29 April 2024 showed progression of the hepatic lesion (PD). The patient was then started on capecitabine (Xeloda) for 3–4 months, which proved unviable, prompting a switch to a regimen of nab-paclitaxel (navelbine) 120 mg on days 1 and 8 and tyrosine kinase inhibitor (tykerb) supplied by Mersal after NGO refusal of tykerb. This regimen was initiated on 8 May 2024. MRI on 30 October 2025 documented a solitary lesion (SOL) for which whole-brain irradiation (WCI) was administered on the same date. The patient continued on tykerb and nab-paclitaxel. A follow-up brain MRI in May 2026 revealed marked radiologic disease (RD) of intraxial SOLs. A repeat CT chest-abdomen-pelvis on 30 August 2026 demonstrated right lower posterior pleural nodularity measuring approximately 19 × 13 mm, appearing stationary, and a hepatic segment 6 focal lesion that had increased from roughly 15 × 15 mm to about 23 × 27 mm (30 × 20 mm as reported), with marginal/central enhancement. The referring team recommended shifting to capecitabine/tykerb with close monitoring of response. The multidisciplinary team concurred and the outcome plan is to proceed with the proposed shift to Xeloda/Tykerb while closely monitoring therapeutic response.	
تحريراً في ,	
التاريخ :	التاريخ : 23-09-2026 الوقت :
الطبيب المعالج :	يعتمد ,



A-Patient ID:

1- Name: داليا عبد محمد عمر

Sex:

MDT form

3- Medical number:
Female

83124529

Old Med:

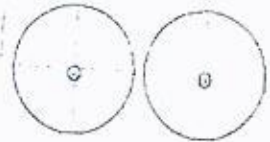
Old Med:

431-2023

2-ID number:

Patient Age: 39 Y

MDT



Family history

Family history

Prostate

PSA

BMC

BMC

REASON OF MDT

REASON OF MDT

marital status

marital status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

Co morbidity

Co morbidity

HTN

Menopausal status

Menopausal status

Initial STAGE

Initial STAGE

25/08/2024

Breast Imaging :

Clinical data

Mets Workup :

BX

IDC

ER

4/8

PR

5/8

Her 2 Neu

3

SISH

Ki67

20

BX COMMENT:
IDC G1, 2019
2023 HFL

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:

SISH COMMENT:

T

2

N

2

M

Liver

Sonomammography

PATIENT HISTORY

2019 PREMENOPUSAL BREAST CANCER --> RT BCS 6/2019 T2 N0 M0,, IDC
RECEIVED 4 AC+ 12 TAXOL + 17 CYCLE HERCEPTIN LAST ONE ON 2/2021
AND WAS ON TAMOXIFEN + ZOLADEX,, RECENTLY PA U/S--> HFL
CTS ON 4/12/2023--> MULTIPLE HFL 3.6X4.5 CM
TRU CUT BIOPSY OF HFL--> MET ADENOCARCINOMA,, ER 4/8 PR -VE ,, HER2 +3
ended taxol x 9 weeks , herceptin x 3 cycles
new ct cap--> DP (29/4) HFL rise size 4.2 x 8 cm
WAS ON TYKERB+ NAVELBine how ever MOH refused to conti. tykerb (full course)
she was clinically responding

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

Continue Tykerb/ Navelbine.

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition



Age / Gender 38 Years - Female

Registered Date 2022-12-18 01:23 م

Reporting Date 2023-01-05 12:03 م

Visit No.

0-1473

Patient Name

دنيا عمر محمد عمر

PATHOLOGY REPORT

Specimen : Tissue blocks from hepatic focal lesion in a known case of breast cancer.

Technique :

Immunostaining effectuated on 4 um thick paraffin sections from the paraffin block, processed in Benchmark: UXT (Ventana) and stained by primary antibodies against DAB as chromogen and hematoxylin as counter stain The sections were treated against: ER, PR, Her-2-neu, Ki-67, GATA3

Result :

- ER: Moderate positive nuclear staining in 10% of tumor cells (Allred score 4/8).
- PR: Weak positive nuclear staining in 5% of tumor cells (Allred score 2/8).
- Her2-neu: Strong complete membranous staining of 80% of tumor cells (score 3+).
- Ki67: Low proliferation index 15%.
- GATA3: Positive nuclear staining of neoplastic cells.

Diagnosis :

Hepatic focal lesion:

Immunohistochemical results confirm the previous diagnosis of metastatic adenocarcinoma of breast origin.

Fatma El-Gendy

Signature

Prof. Dr. Nehal Radwan

DR. Peng Ahmed

4/1/2023



Patient Name: Donia Omar Mohamed Omar	Date : 04/05/2025
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID: 431/2023	Pat. ID: 32936

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

****As compared to the previous study dated 2-2-2025, the following study shows:**

I Chest:

- Status post right BCS showing **rather stationary** appearance of the right UIQ irregular density likely scaring.
- No pathological axillary lymph nodes.
- No pulmonary masses, nodules, consolidation, cavitation or collapse
- No hilar or mediastinal enlarged lymph nodes.
- No evidence of pleural effusion, or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- **Still noted** Left sided porta A Cath.

II Abdomen & pelvis:

- **Still noted** enlarged liver with diffuse homogeneous low CT parenchymal attenuation denoting fatty infiltration with still noted focal area of isodense attenuation at segment VI of right lobe.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal enhanced CT appearance of the pancreas, aorta, I.V.C and both suprarenal's.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

Pat. Name: Donia Omar Mohamed Omar	Pat. ID: 1411557486
Date : 15/09/2024	

Nuclear Medicine Unit

ISOTOPIC BONE SCAN REPORT

Clinical History:

39-Y/O lady; history of right breast cancer; referred for metastatic workup.

TECHNIQUE:

Bone scintigraphy done 2 hrs. Following I.V injection of 20 mCi of Tc-99m M.D.P.

FINDINGS:

- The scanned skeleton is free from any definite scintigraphic evidence of abnormal active or cold focal bony lesions that may account for metastatic bone disease.
- Degenerative changes are noticed at the spine, especially the lower lumbar vertebrae.
- Osteoarthritic changes are noticed over the big joints, especially both knees.

IMPRESSION:

- The scanned skeleton is currently scintigraphically free from metastases.

Thanks for reference,

Dr/Ahmed NASR, M.D.

Consultant of Nuclear Medicine

(AN)

"For Any Inquiries regarding the findings in the report please contact us on
radreport@technoscan.com.eg"



Patient Name: Donia Omar Mohamed Omar	Date : 21/01/2024
Age : 38 Year(s)	Patient ID: 32936
Exam : Item 1	

MRI EXAMINATION OF THE DORSAL SPINE:

Technique of examination:

- Sagittal T1 & T2 WIs
- Axial T1 & T2 WIs
- Post Gadolinium DTPA Axial, & Sagittal T1 WIs

**** A reported case of MBC,**

MRI Findings:

- Few small areas of altered marrow signals are seen implicating the D4 and D7 vertebral bodies eliciting low signals in all pulse sequences with no appreciable enhancement in the post gadolinium series. No associated soft tissue components.
- Preserved dorsal curvature.
- Normal diameters of the bony spinal canal.
- No evidence of disc herniation or significant bulges.
- Normal MRI appearance of the dorsal cord with no evidence of cord edema, malacia or syrinx formation.
- No paraspinal signal abnormalities.

Conclusion:

Few D4 and D7 small marrow focal lesions, considering the patient history of Cancer breast the possibility of metastatic deposits is to be considered for correlation with the Bone Scan Findings.

Dr. Alaa R. Hani, M.Sc., FEBR

Dr. Amr Farouk, M.D.



جمهورية الكويت
بطاقة تعريف الشخصية

دنيا

عسر محمد عسر

١١ شى عزالرحمن عسر

الجزء - الجزء

١٩٨٥/٠١/١١

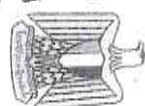
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KC3436581



٢٠٢٣/٠٨

٢٨٥٠٦١١٢١٠٠٢٠٩
اخصائية تحليل طبيه



متروجه

أشئ مسلمة
محمد نور الدين محمد السيد

البطاقة سارية حتى ٢٠٣٠/٠٨/٠٩



جوسناف
رئيس
مجلسمبادرة رئيس الجمهورية
لدعم صحة المرأة المصريةالهيئة العامة
للحساسية
والأمراض

09/16/2026

MDT FORM

Name:	Donia Omar Mohamed .	MRN:	437
Age:	41 Y	ID:	28506112100209
Menopausal Sta	premenopausal	Phone:*	01093680205
NRS SCORE:		Marital Status:	married with one daughter
BMI:		Family History:	+ve FH (her aunt , breast cancer)
Gender		Reason Of MDT:	
CO Morbidity:	no	progressive	
Surgical History:*	RT BCS	ECOG Performance Status	☐ 0 ☒ 1 ☐ 2 ☐ 3 ☐ 4

Clinical Data:

BX:
ER: PR: HER2: KI67:

History:

2019 PREMENOPASUAL RT BREAST CANCER >>>>RT BCS 6/2019
>>>>T2N0M0 IDC GIII ER 4/8 PR 5/8 HER2+3 KI6720% RECEIVED
4AC+12 TAXOL +17 HERCEPTIN LAST ONE AT 2/2021
TAMOXIFEN+ZOLADEX TILL 12/2023
then >>> HFL BIOPSY MET ADENOCARCINOMA >> ER 4/8 PR -VE
HER2+3 received TAXOL x9 +3 HERCEPTIN
NEW CT CAP DP 29/4 HFL PD
received xeloda x 3-4 months months >> was unviable so shifted to
navelbine , tykerb
MDT AT 8/2024 NAVELBINE+TYKERB on navelbine 120 mg D1,D8 tykeb
from Mersal (As NGO refused tykerb patient recieved full course)
MRI >> SOL >> recieved WCI 30/10 (9/2025)
continued on tykerb , navelbine
new MRI brain 5/2026 > marked RD of intraxial SOLs.
CT CAP 30/08/2026 reviewed vs previous:
Chest: R lower post. pleural nodularity, max ~19x13 mm ? stationary.
.....Liver: S6 focal lesion with interval ? size, ~15x15 mm ? progressed to
~23x27 mm (30x20 mm in body of report), with marginal/central
enhancement.

Sonomammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

For shifting to Xeloda/Tykerb with close monitoring of response.

Clinical Staging:

Pathological Staging:

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د/نبيل لاشين	د/احمد عيسى		

about:blank

Patient's Name: Dunia Omar Mohammed
Date: 2026-08-30
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE CONTRAST ENHANCED C.T. EXAMINATION OF THE CHEST,
 ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Known case of breast cancer.
- Left breast porta cath, its tip noted in SVC.
- Few left axillary lymph nodes largest 9 mm .
- Still noted Right lower posterior pleural nodularity ,largest measures about (19x13mm) in AP and transverse diameter.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Mild enlarged liver showing homogenous parenchymal attenuation. Progressive course according to the right lobe segment VI lesion measuring 30× 20 mm show marginal and central enhancement. That was 15x 15 mm
- No lesion detected in the left lobe.
- No dilated biliary radicles.
- Both kidneys are normal in site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T. appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- IUD is seen in place

Opinion:

- ❖ Known case of breast cancer.
regarding last study :
- ❖ progressive course : increase size of hepatic focal lesion from (15x15mm) to (23x27mm).
- ❖ Stationary course regarding pleural nodularity in right side .

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيند /

تحية طيبة وبعد،،

إيماء للطلاب المقدم من بشأن تقرير طبي للمريض: **دنيا كهر محمد كهر**

بقسم رقم طبي

نشر ف بأن فوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد انه:

السنه كهر محمد كهره كى ربه نهائى عسأوا، **١** سر طينه الكثرى وناخو حاست
١٢ كسجج دجج رجت نجا نسجج نكور بالبراج الا نسجج واله جج
والله يسر وسيسر وودر صلاجه وسر عجة كحسقة بالهزى العلوى
بالطبيب و تحتاج الى استكمال الفحوصات العلوية بسركا بس
و **١٠** خط و علاج نسجج كسجج كسجج الى الله

تحريراً في

الوقت: التاريخ: **٢٠٢٩/٩/١٨**

الطبيب المعالج
دكتور محمد كهر

يتمد ،،

جمهورية مصر العربية
بطاقة تعريف الشخصية



دنيا
عمر محمد عمر
١١ شى عبدالرحمن غير
الجزء - الجزء

١٩٨٥/٠١/١١

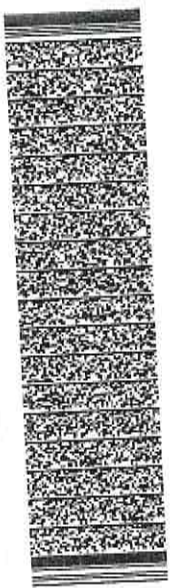
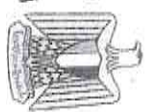
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KC3436581

٢٠٢٣/٠٨

٢٨٥٠٦١١٢١٠٠٢٠٩
إخصائية تحليل طبيه

متروجة
أشئ مسلمة
محمد نور الدين محمد السيد
البطاقة سارية حتى ٢٠٣٠/٠٨/٠٩



جوسناف
رئيس
فريقالخط
9-17مبادرة رئيس الجمهورية
لدعم صحة المرأة المصريةالهيئة العامة
للحساسية
والأمراض

09/16/2026

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new MRI brain 5/2026 > marked RD of intraxial SOLs.
CT CAP 30/08/2026 reviewed vs previous:
Chest: R lower post. pleural nodularity, max ~19x13 mm ? stationary.
.....Liver: S6 focal lesion with interval ? size, ~15x15 mm ? progressed to
~23x27 mm (30x20 mm in body of report), with marginal/central
enhancement.

Sonomammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

For shifting to Xeloda/Tykerb with close monitoring of response.

Clinical Staging:

Pathological Staging:

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د/نبيل لاشين	د/احمد عيسى		

about:blank

Patient's Name: Dunia Omar Mohammed
Date: 2026-08-30
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE CONTRAST ENHANCED C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:

I-CHEST:

- Known case of breast cancer.
- Left breast porta cath, its tip noted in SVC.
- Few left axillary lymph nodes largest 9 mm .
- Still noted Right lower posterior pleural nodularity ,largest measures about (19x13mm) in AP and transverse diameter.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Mild enlarged liver showing homogenous parenchymal attenuation. Progressive course according to the right lobe segment VI lesion measuring 30× 20 mm show marginal and central enhancement. That was 15x 15 mm
- No lesion detected in the left lobe.
- No dilated biliary radicles.
- Both kidneys are normal in site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T. appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- IUD is seen in place

Opinion:

- ❖ Known case of breast cancer.
regarding last study :
- ❖ progressive course : increase size of hepatic focal lesion from (15x15mm) to (23x27mm).
- ❖ Stationary course regarding pleural nodularity in right side .